

Distribution separates arterial and venous events

Retinal pallor suggests ischaemia; congested veins and haemorrhage suggest a venous occlusion.

Territory and appearance

Event	Fundus clue
CRAO	Diffuse whitening and a cherry red spot.
BRAO	Sectoral retinal whitening.
CRVO	Widespread haemorrhage with congested tortuous veins.
BRVO	Venous haemorrhage limited to one branch territory.

Clinical meaning

Poor acuity and a marked RAPD in CRVO suggest greater ischaemia; fluorescein angiography assesses capillary non-perfusion. Macular oedema and neovascular complications threaten vision. Review vascular risks, and consider directed blood, prothrombotic or inflammatory assessment in younger patients without the usual risk factors.

An arterial warning can remain important after recovery

Transient monocular visual loss of embolic pattern warrants urgent systemic vascular assessment.

Embolic monocular visual loss

Recognise sudden persistent loss or transient episodes



Arrange urgent ophthalmic and systemic assessment



Investigate carotid and cardiac embolic sources



Address vascular risks and suspected inflammatory causes

Clinical meaning

Recovery after amaurosis fugax does not remove stroke risk. In CRAO, a cilioretinal supply may preserve central vision. Hypertensive grading progresses from vessel narrowing and AV nicking to haemorrhages and exudates; adding disc swelling gives grade 4. Check blood pressure when interpreting this pattern.